2019

Medical Compliance Training: CPCO™

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Chapter 1

A History of Healthcare Compliance in the United States

As a Certified Professional compliance officer (CPCO™), it will be necessary for you to be aware of the history of healthcare compliance in the United States and be familiar with current healthcare regulations. You will not be required to memorize all the regulations; however, it will be important for you to know where resources are located.

The objectives for Chapter 1 are:

- Forming an understanding of the history of healthcare compliance;
- Understanding the key agencies involved in healthcare compliance development and enforcement;
- Demonstrating an understanding of the key requirements needed to effectively develop, implement, and monitor a healthcare compliance program, based on governmental regulatory guidelines.

In 1992, the General Accounting Office (GAO) identified Medicare claims to be at high risk for fraud and abuse (GAO/HR-93-6, Dec. 1992). Subsequent to this determination, in 1996 the Office of Inspector General (OIG) initiated an audit of the Healthcare Finance Administration (later renamed Centers for Medicare & Medicaid Services [CMS]) Medicare claims payment system. This resulted in an estimated finding of over $23 billion in improper payments, and the beginning of intensified provider audits and the need for compliance.

The first major audit targets were teaching hospitals, such as the University of Pennsylvania. The investigation of the University of Pennsylvania resulted in a settlement with the Department of Justice (DOJ) for $30 million (without admitting any wrongdoing). This case is often referred to as Clinical Practices of the University of Pennsylvania (CPUP). An audit identified that some of the university's teaching physicians had inappropriately billed Medicare because the medical records did not sufficiently document their involvement in services that were provided by resident physicians. The audit also determined that some of the teaching physicians had up-coded their claims. (Up-coded: Billed for more complex and more expensive services than may have been provided). These audits are commonly known as Physician at Teaching Hospital (PATH) audits. The PATH audits targeted insufficient medical record documentation to support whether a physician either performed the service or was present when a resident or fellow performed the service. As a result of findings in the 1995 CPUP audit, insufficient documentation supporting the level of service was also targeted.

The OIG and the DOJ created a nationwide initiative to determine if compliance with the Medicare billing rules was being adhered to by other teaching hospitals. Other initiatives followed the PATH audits. Operation Bad Bundle involved clinical laboratories, durable medical equipment (DME) fraud initiatives, hospice and home health fraud initiatives, and many more.

The OIG created the first compliance guidance document for hospitals in February 1998. Similar compliance guidance documents were issued for other sectors of the healthcare industry beginning in August 1998. The list of sectors and dates are below. The compliance guidance documents can be found on the OIG website at: https://oig.hhs.gov/compliance/compliance-guidance/

02-23-1998

Compliance Program Guidance for Hospitals (63 FR 8987; Feb. 23, 1998)

Supplemental Compliance Program Guidance for Hospitals (70 FR 4858; Jan. 31, 2005)

08-07-1998

Compliance Program Guidance for Home Health Agencies (63 FR 42410; Aug. 7, 1998)

08-24-1998

Compliance Program Guidance for Clinical Laboratories (63 FR 45076; Aug. 24, 1998)

12-18-1998

Compliance Program Guidance for Third-Party Medical Billing Companies (63 FR 70138; Dec. 18, 1998)

07-06-1999

Compliance Program Guidance for the Durable Medical Equipment, Prosthetics, Orthotics, and Supply Industry (64 FR 36368; July 6, 1999)

10-05-1999

Compliance Program Guidance for Hospices (64 FR 54031; Oct. 5, 1999)

11-15-1999

Compliance Program Guidance for Medicare+ Choice Organizations (64 FR 61893; Nov. 15, 1999)

03-16-2000

Compliance Program Guidance for Nursing Facilities (65 FR 14289; March 16, 2000)

Supplemental Compliance Program Guidance for Nursing Facilities (73 FR 56832; Sept. 30, 2008)

03-24-2003

Compliance Program Guidance for Ambulance Suppliers (68 FR 14245; March 24, 2003)

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10-05-2000

Compliance Program Guidance for Individual and Small Group Physician Practices (65 FR 59434; Oct. 5, 2000)

05-05-2003

Compliance Program Guidance for Pharmaceutical Manufacturers (68 FR 23731; May 5, 2003)

11-28-2005

Draft Compliance Program Guidance for Recipients of PHS Research Awards (70 FR 71312; Nov. 28, 2005)

NSTC Launches Government-Wide Initiative Based on OIG Draft Guidance for HHS Research Grants (June 7, 2006)

Another compliance initiative by the federal government can be found in the Patient Protection and Affordable Care Act or, ACA, as amended seven days later by the Healthcare and Education Reconciliation Act of 2010 (collectively referred to as Healthcare Reform Law). Sections 6102 and 6401 in the ACA mandate a broad range of providers, suppliers, and physicians to adopt a compliance and ethics program.

In 2016, the final rule for the Medicare Access and CHIP Reauthorization Act (MACRA) was released. This regulation ended the Sustainable Growth Rate (SGR) formula and created the Quality Payment Program (QPP) to place a higher focus on quality of care and patient outcomes.

Additional Compliance Requirements

Healthcare billing and reimbursement compliance is not the only type of regulation that must be followed by providers. HIPAA, which has fraud and abuse provisions, also has mandatory privacy and security compliance program requirements overseen by both the Office of Civil Rights (OCR) and the U.S. Department of Health and Human Services (HHS). The Department of Labor (DOL) requires compliance with employee and employer laws including the Fair Labor Standards Act (FLSA), the Occupational Safety and Health Act (OSHA), the Civil Rights Act (CRA), and the employment provisions of the Employee Retirement and Income Security Act (ERISA). Laboratories are regulated and overseen by CMS and by the Centers for Disease Control and Prevention (CDC). There are even more agencies and regulations that affect health providers of all sizes. Entities and their compliance officers must understand the laws that regulate their workplaces and responsibilities.

Office of Inspector General (OIG)

Since its establishment in 1976, the OIG has been at the forefront of the nation's efforts to fight fraud, abuse, and waste in Medicare, Medicaid, and more than 300 other HHS programs. The OIG's mission is to protect the integrity of HHS programs as well as the health and welfare of program beneficiaries. The OIG is the largest inspector general's office in the federal government, with more than 1,700 workers dedicated to combating fraud, waste, and abuse. Its goal is to improve the efficiency of HHS programs.

A majority of the OIG's resources goes toward the oversight of the Medicare and Medicaid programs - meant to help this country's most vulnerable citizens (those either disabled, underprivileged, or over the age of 65) that also happen to represent a significant part of the federal budget.

OIG's oversight extends to programs under other HHS agencies, including Center for Disease Control and Prevention (CDC), National Institutes of Health (NIH), and the U.S. Food and Drug Administration (FDA).

OIG develops and distributes resources to assist the healthcare industry in its efforts to comply with the nation's fraud and abuse laws and to educate the public about fraudulent schemes so they can protect themselves and report suspicious activities. The OIG consists of six departments:

1. Immediate OIG—responsible for the overall fulfillment of the OIG's mission and for promoting effective management and quality of the agency's processes and products.
2. Office of Audit Services—performs independent audits of HHS programs and/or HHS grantees and contractors to examine their performance.
3. Office of Evaluation and Inspections—conducts national evaluations of HHS programs from a broad issue-based perspective.
4. Office of Management and Policy—provides mission and administrative support to the OIG.
5. Office of Investigations (OI)—conducts criminal, civil, and administrative investigations of fraud and misconduct related to HHS programs, operations, and beneficiaries. The OI also operates an OIG hotline.
6. Office of Counsel to the Inspector General—provides legal advocacy and counsel to the Inspector General and OIG's other components.

Each department is comprised of individuals who perform a nationwide network of audits, investigations, and evaluations, which result in timely information, as well as cost-saving or policy recommendations for decision-makers and the public. This network also assists in the development of cases for criminal, civil, and administrative enforcement.

Department of Justice (DOJ)

The DOJ works closely with the OIG. It is comprised of more than 105,000 employees, including hundreds of lawyers who

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enforce the nation's federal criminal laws and help to develop and implement criminal law policies. Their role in healthcare fraud and healthcare compliance is to investigate and prosecute healthcare crimes.

The DOJ consists of numerous agencies, including the Federal Bureau of Investigation (FBI) and the Drug Enforcement Administration (DEA). A list of all the agencies can be found at: http://www.justice.gov/agencies. The DOJ's current strategic plan (2018-2022) includes the following overarching goals:

- Enhance National Security and Counter the Threat of Terrorism;
- Secure the Borders and Enhance Immigration Enforcement and Adjudication;
- Reduce Violent Crime and Promote Public Safety; and
- Promote Rule of Law, Integrity, and Good Government.

Corporate Integrity Agreements (CIAs) and Certification of Compliance Agreements (CCAs)

The OIG uses CIAs to enforce compliance within a healthcare organization. CIAs are usually used as a part of a civil settlement with a provider or organization when there has been an investigation into the claims submitted to a federal payer. It can also be used when an individual or entity has been found guilty of defrauding a federal healthcare program, including Medicare and Medicaid. "Providers or entities agree to the obligations, and in exchange, the OIG agrees not to seek their exclusion from participation in Medicare, Medicaid, or other Federal healthcare programs." (https://oig.hhs.gov/compliance/corporate-integrity-agreements/). The core elements of CIAs were created around the 1995 Federal Sentencing Guidelines (https://oig.hhs.gov/publications/docs/retrospective/AnniversaryPub.pdf). CIAs are usually only offered when the OIG believes the provider(s) can be compliant with the program regulations.

Why submit to this type of consequence? Because the alternative is being excluded from the federal healthcare programs. Congress mandated the exclusion of physicians and other providers convicted of crimes from participation in Medicare and Medicaid programs in 1977. In 1981, Congress also enacted the Civil Monetary Penalties Law (CMPL) to address healthcare fraud and abuse. The CMPL allowed the OIG to impose CMPs and program exclusions against individuals and entities who submit false claims to Medicare or Medicaid. The word "excluded" means that the individual or entity cannot work with, and are prohibited from participation in, all programs that deal with state or federal dollars (https://oig.hhs.gov/exclusions/effects_of_exclusion.asp).

"Protecting Public Health and Human Services Programs; A 30-Year Retrospective" reviews some of the history of CIAs. The OIG executed its first CIAs in the mid-1990s. These initial CIAs only required the provider to attend training and certify such to the OIG. As CIAs gained popularity over the next decade, the OIG added "integrity" provisions and required that the provider establish an effective compliance program following the practices set forth in the guidelines.

Inspector General Daniel Levinson disseminated *An Open Letter to Healthcare Providers* in 2006, in which he discussed the current enforcement and noted that, "many providers have independently developed robust and effective compliance programs, which include internal auditing mechanisms. In appropriate cases, we have agreed to reduce the obligation on providers settling healthcare fraud matters by entering into Certification of Compliance Agreements (CCAs), rather than more extensive CIAs. CCAs require providers to certify that they will continue to operate their existing compliance programs for a fixed term, typically 3 years, rather than enter into a more extensive CIA with a 5-year term. CCAs do not require independent review organizations (IROs) to conduct or verify audits or claims reviews." CIAs are discussed in further detail in Chapter 9 of this curriculum.

Office of Civil Rights (OCR)

Federal civil rights laws and the HIPAA Privacy Rule protect our fundamental rights of nondiscrimination and health information privacy. Civil rights help to protect those residing in the United States from unfair treatment or discrimination in receiving or accessing services because of race, color, national origin, disability, age, sex (gender), or religion.

(Note: Employment discrimination is overseen by the Department of Labor [DOL].)

The OCR is responsible for enforcing the HIPAA Privacy Rule and Security Rule provisions. It also oversees the provisions of: the Civil Rights Act of 1964 as it applies to Limited English Proficiency (see below), the confidentiality provisions of the Patient Safety Act and Rule, the Church Amendments, section 245 of the Public Health Service Act, and the Weldon Amendment. These statutes are collectively known as the Federal Healthcare Provider Conscience Protection Laws.

The OCR helps protect individuals from discrimination in certain healthcare and social service programs. Some of these programs include:

- Hospitals, health clinics, nursing homes
- Medicaid and Medicare agencies
- Welfare programs
- Daycare centers
- Doctors' offices and pharmacies

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- Children's health programs
- Alcohol and drug treatment centers
- Adoption agencies
- Mental health and developmental disabilities agencies

The OCR protects citizens of the United States by:

- Teaching health and social service workers about civil rights, health information privacy, and patient safety confidentiality laws they must follow.
- Educating communities about civil rights and health information privacy rights.
- Investigating civil rights, health information privacy, and patient safety confidentiality complaints to find out if there is discrimination or violation of the law and taking action to correct problems.

Limited English Proficiency (LEP)

All entities, including healthcare providers, that receive federal financial assistance from HHS, either directly or indirectly, are covered by LEP requirements. On August 11, 2000, former President Clinton signed Executive Order 13166. This order requires agencies and programs to take steps to ensure federally-funded activities are accessible to all persons who, because of national origin, are not proficient or are limited in their ability to communicate in the English language.

HHS submitted "*LEP Strategic Plan to Improve Access to HHS Programs and Activities by Limited English Proficient (LEP) Persons*" to the DOJ, December 12, 2000, as was required by the order. In November 2003, HHS published revised policy guidance to assist healthcare professionals and organizations to meet their Title VI obligation of non-discrimination on the basis of race, color, or national origin (https://www.hhs.gov/civil-rights/for-individuals/special-topics/limited-english-proficiency/guidance-federal-financial-assistance-recipients-title-VI/index.html).

All entities that receive federal financial assistance from HHS, either directly or indirectly, through a sub grant or subcontract, are covered by this guidance. Recipients of HHS assistance may include:

- Hospitals, nursing homes, home health agencies, and managed care organizations
- Universities and other entities with health or social service research programs
- State, county, and local health agencies
- State Medicaid agencies
- State, county, and local welfare agencies
- Programs for families, youth, and children
- Head Start programs
- Public and private contractors, subcontractors, and vendors
- Physicians and other providers who receive federal financial assistance from HHS

An LEP person/patient who needs an interpreter to translate to and from the person's primary language must be provided at no cost to the patient. An LEP person may also need documents written in English translated into his or her primary language so that person can understand important documents such as his or her treatment plan, Notice of Privacy Practices, medication list, etc. The information regarding the specific requirements can be found at https://www.hhs.gov/civil-rights/for-individuals/special-topics/limited-english-proficiency/index.html and should be made part of each healthcare entity's compliance program.

Know the Difference between Fraud and Abuse

Fraud has several definitions. Under HIPAA, fraud is defined as "knowingly, and willfully executing or attempting to execute a scheme to defraud any healthcare benefit program." The key term is *knowingly*. CMS defines fraud in a Medicare Learning Network product, "Medicare Fraud & Abuse: Prevention, Detection, and Reporting," that Medicare fraud is typically characterized by:

- "Knowingly submitting false statements or making misrepresentations of fact to obtain a federal healthcare payment for which no entitlement would otherwise exist;
- Knowingly soliciting, paying, and/or accepting remuneration to induce or reward referrals for items or services reimbursed by federal healthcare programs; or
- Making prohibited referrals for certain designated health services."

CMS goes on to define abuse as "practices that, either directly or indirectly, result in unnecessary costs to the Medicare Program."

Abuse is similar to fraud, except that the investigator cannot establish the act was committed knowingly, willfully, and intentionally. The difference between fraud and abuse is the individual's intent; however, both have the same impact in that they steal valuable resources from the healthcare industry.

Improper payment is a significant element in fraud, waste, and abuse. The following illustrates examples of how common actions in healthcare escalate from a simple mistake to intentional deceptions or fraud. Here are a few examples:

> Example of Fraud: The office manager, or a provider, enters extra codes on a claim form knowing that the extra codes are wrong and will generate extra money for the office.

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Example of Abuse: The physician documents and bills a 99213 every two weeks for patients when it is only medically necessary to see them once a month.

Example of Waste: The provider orders a lab on a patient that has many various tests involved (called a lab panel) instead of just ordering 1 lab test that is was needed.

(https://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/Avoiding_Medicare_FandA_Physicians_FactSheet_905645.pdf)

OIG's Five-principle Strategy to Combat Healthcare Fraud, Waste, and Abuse

June 25, 2009, Inspector General Daniel Levinson testified to the House Committee on Energy and Commerce, Subcommittee on Health. During this testimony, Levinson described the OIG's strategy to combat healthcare fraud, waste, and abuse. Part of his testimony is reprinted below:

- "Collaboration and innovation are essential in the fight against fraud. On May 20, 2009, HHS Secretary Kathleen Sebelius and Attorney General Eric Holder announced a new initiative to marshal significant resources across the government to prevent healthcare waste, fraud, and abuse; crack down on fraud perpetrators; and enhance existing partnerships between HHS and DOJ to reduce fraud and recover taxpayer dollars.
- For federal healthcare programs to best serve beneficiaries and remain solvent for future generations, the government must pursue a comprehensive strategy to prevent, detect, and remediate fraud, waste, and abuse. Based on OIG's extensive experience in combating healthcare fraud, waste, and abuse, we have identified the following five principles that we believe should guide the development of any national healthcare integrity strategy.
- We believe that our "Five Principles" strategy provides the framework to identify new ways to protect the integrity of the programs, meet the needs of beneficiaries, and keep federal healthcare programs solvent for future generations."

Note: The "Five Principles" strategy was originally recommended in 1990 at a hearing for Federal Financial Management, Government Information, Federal Services, and International Security Subcommittee of the Committee on Homeland Security and Governmental Affairs, U.S. Senate. Transcripts of this testimony can be found at: http://www.gpo.gov/fdsys/pkg/CHRG-111shrg50390/html/CHRG-111shrg50390.htm.

OIG's Five-principle Strategy to Combat Healthcare Fraud, Waste, and Abuse

1. **Enrollment**—Scrutinize individuals and entities that want to participate as providers and suppliers prior to their enrollment in healthcare programs.

   OIG Recommendation: Providers and suppliers applying for enrollment in Medicare or Medicaid should be screened before they are granted billing privileges. Heightened screening measures for high-risk items and services could include requiring providers to meet accreditation standards, requiring proof of business integrity or surety bonds, periodic recertification, and onsite verification that conditions of participation have been met, and full disclosure of ownership and control interests. The cost of this screening could be covered by charging application fees. New providers and suppliers should also be subject to a provisional period during which they are subject to enhanced oversight, such as prepayment review and payment caps.

2. **Payment**—Establish payment methodologies that are reasonable and responsive to changes in the marketplace.

   OIG Recommendation: All payment methodologies create incentives and fraud risks that should be identified and addressed. For example, fee-for-service payments create financial incentives to maximize the number and complexity of services provided, even when such services are not medically necessary. Conversely, under a fixed prospective payment system, financial incentives encourage fewer services and patients may not receive all the care they need and for which the program is paying. In considering any payment structure, it is imperative to identify the incentives it creates, and associated risks, and to implement necessary safeguards to remediate the negative incentives and reduce fraud risks.

3. **Compliance**—Assist healthcare providers and suppliers in adopting practices that promote compliance with program requirements, including quality and safety standards.

   OIG Recommendation: Although compliance programs do not guarantee reduced fraud and abuse, they are an important component of a comprehensive government-industry partnership to promote program integrity. We advocate that providers and suppliers be required to adopt compliance programs as a condition of participating in the Medicare and Medicaid programs. Further, the obligation of providers and suppliers to repay overpayments they discover through

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compliance efforts or otherwise should be made explicit in the statute. There should be no question that providers and suppliers must return taxpayer dollars they should not have received in the first place.

4. **Oversight**—Vigilantly monitor programs for evidence of fraud, waste, and abuse.

   OIG Recommendation: Needed improvements in program oversight include real-time access to data for law enforcement; uniform, comprehensive data elements; more timely collection and validation of data; robust reporting of data by tates and others; interoperability of systems; consistent data extraction methods; and the ability to draw and analyze claims and provider data across Medicare Parts A, B, C, D, and Medicaid. CMS is building an Integrated Data Repository (IDR) that will, when completed, contain a wealth of data across several programs. Although the system is still under development, the prospect of such a comprehensive data warehouse holds considerable promise for detecting and preventing fraud and abuse.

   In addition, we advocate the consolidation and expansion of the various provider databases, including the Healthcare Integrity and Protection Data Bank (HIPDB), the National Practitioner Data Bank (NPDB), and OIG's List of Excluded Individuals/Entities (LEIE). Providing a centralized, comprehensive, and public database of adverse actions and other sanctions – including a national registry of patient abuse and neglect – would be an effective means of preventing providers and suppliers with problem backgrounds from moving from state to state unnoticed by licensing, government, and health plan officials.

5. **Response**—Respond swiftly to detected fraud, impose sufficient punishment to deter others, and promptly remedy program vulnerabilities.

   OIG Recommendation: Even the best antifraud efforts are ineffective if fraud is not promptly detected and, once detected, promptly punished and deterred. For example, our investigations have found evidence of an increase in organized crime in healthcare. Healthcare fraud is attractive to organized crime because the penalties are lower than those for other organized-crime-related offenses (eg, offenses related to illegal drugs); there are low barriers to entry (eg, a criminal can easily obtain a supplier number, gather some beneficiary numbers, and bill the program); schemes are easily replicated; and detection efforts often are hampered by lack of access to real-time data. We need to alter the cost-benefit analysis by increasing the risk of swift detection and the certainty of punishment.

   In addition, it is difficult to stop the flow of Medicare dollars to criminals who are under investigation for known healthcare fraud schemes. An explicit payment suspension authority would enable Medicare to keep taxpayer dollars out of the pockets of criminals in cases where the government has credible evidence of fraud. These criminals often take the money and disappear before the government can complete an investigation and prosecute them. An explicit payment suspension authority is a critical, money-saving tool in these situations.

   OIG currently uses a range of administrative sanctions, including civil monetary penalties (CMP) and program exclusions, as an adjunct to criminal and civil enforcement. However, OIG has identified a number of enhancements to these administrative authorities that would increase our ability to address emerging schemes, such as authorizing CMPs for false provider enrollment applications and for the ordering or prescribing of items or services by an excluded entity. Amending the law to align our CMP authorities with the recent False Claims Act amendments would also be helpful.

COMPLIANCE TIP

Exclusions are imposed for a number of reasons:

Mandatory exclusions: OIG is required by law to exclude from participation in all Federal healthcare programs individuals and entities convicted of the following types of criminal offenses: Medicare or Medicaid fraud, as well as any other offenses related to the delivery of items or services under Medicare, Medicaid, SCHIP, or other State healthcare programs; patient abuse or neglect; felony convictions for other healthcare-related fraud, theft, or other financial misconduct; and felony convictions relating to unlawful manufacture, distribution, prescription, or dispensing of controlled substances.

Permissive exclusions: OIG has discretion to exclude individuals and entities on a number of grounds, including (but not limited to) misdemeanor convictions related to healthcare fraud other than Medicare or a State health program, fraud in a program (other than a healthcare program) funded by any Federal, State or local government agency; misdemeanor convictions relating to the unlawful manufacture, distribution, prescription, or dispensing of controlled substances; suspension, revocation, or surrender of a license to provide healthcare for reasons bearing on professional competence, professional performance, or financial integrity; provision of unnecessary or substandard services; submission of false or fraudulent claims to a Federal healthcare program; engaging in unlawful kickback arrangements; defaulting on health education loan or scholarship obligations; and controlling a sanctioned entity as an owner, officer, or managing employee.

Source: https://oig.hhs.gov/exclusions/background.asp

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Section Review 1.1

1. Sue works for ABC Family Physicians. The providers at this office ask her to research the department that helps protect patients from unfair treatment or discrimination. What department or agency would that be?

    a. Equality in Employment Agency
    b. Office for Civil Rights
    c. Department of Justice
    d. Office of Inspector General

2. Which government department is comprised of thousands of employees who enforce the nation's federal criminal laws and help develop and implement criminal law policies?

    a. Office of Inspector General
    b. Centers for Medicare & Medicaid Services
    c. Healthcare Lawyers Association
    d. Department of Justice

3. Which department is the largest inspector general's office in the federal government?

    a. Office of Inspector General
    b. U.S. Department of Health & Human Services
    c. Department of Justice
    d. Centers for Medicare & Medicaid Services

4. What term would be used for a practice that, either directly or indirectly, results in unnecessary costs to the Medicare program?

    a. Fraud
    b. Mistake
    c. Waste
    d. Abuse

5. Which federal government department is the OIG NOT responsible for overseeing?

    a. Centers for Medicare & Medicaid Services
    b. Centers for Disease Control and Prevention
    c. U.S. Food & Drug Administration
    d. Drug Enforcement Agency

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Model Compliance Plans

Health and Human Services (HHS), the executive branch of the government, the legislative branch of the government, and the judicial branch of the government have made it clear: Healthcare entities must comply with the laws … or else. The OIG has provided the compliance guidance documents, letters, advisory opinions, and fraud alerts published and posted to the government websites, as well as copies of actual CIA documents to assist providers and their staff in developing and implementing their own compliance programs.

Compliance professionals need to be creative when implementing their entity's compliance program. The compliance program needs to be tailor-fit to the needs of the individual practice or healthcare organization. The OIG has stated that an effective compliance program can help create financial success, customer loyalty, community support, and employee satisfaction.

As you start to read the OIG guidance documents, you will find there is a fine line between compliance and ethics, and they are often intertwined. Even though there is not an enforcement date for a required compliance plan, it makes good business sense. It helps to create a culture of ethical expectations and compliance within any physician practice or other medical organization.

Each of the compliance guidance documents already published, as well as information already published regarding the mandatory compliance program requirements, state that the Federal Sentencing Guidelines (FSGs) are and will be used for any future guidance. There are seven elements in these guidelines the OIG has stated are necessary in an effective compliance program:

1. Conducting internal monitoring and auditing
2. Implementing compliance and practice standards
3. Designating a compliance officer or contact
4. Conducting appropriate training and education
5. Responding appropriately to detected offenses and developing and implementing corrective action
6. Developing open lines of communication
7. Enforcing disciplinary standards through well-publicized guidelines

(These seven elements are discussed in detail in Chapter 2.)

The compliance program guidance is recognized by the United States Sentencing Commission (USSC). These seven core elements are used by the USSC as the basis for the FSGs for healthcare offenses.

Compliance Professionals

It is important for compliance officers to understand what a compliance program is NOT. It is not just a book or set of documents collecting dust on a shelf. It is not a list or folder containing chart audit results. It is not an exercise to placate the government, and it certainly is NOT a mandate for providers to change the way they practice medicine.

A compliance program is the process each organization implements to achieve the stated goals of the organization. The program should encompass all areas of regulation applicable to the medical organization. In order for a compliance program to be effective, it will need to be reviewed and updated regularly.

COMPLIANCE TIP

February 8, 2017, the U.S. Department of Justice published guidance on the Evaluation of Corporate Compliance Programs.

The document begins with:

"The Principles of Federal Prosecution of Business Organizations in the United States Attorney's Manual describe specific factors that prosecutors should consider in conducting an investigation of a corporate entity, determining whether to bring charges, and negotiating plea or other agreements. These factors, commonly known as the "Filip Factors," include "the existence and effectiveness of the corporation's pre-existing compliance program" and the corporation's remedial efforts "to implement an effective corporate compliance program or to improve an existing one." "

Important topics and sample questions found relevant in evaluating corporation compliance plans include:

1. Analysis and Remediation of Underlying Misconduct
2. Senior and Middle Management
3. Autonomy and Resources
4. Policies and Procedures
5. Risk Assessment
6. Training and Communications
7. Confidential Reporting and Investigation
8. Incentives and Disciplinary Measures
9. Continuous Improvement, Periodic Testing and Review
10. Third Party Management
11. Mergers and Acquisitions

Source: https://www.justice.gov/criminal-fraud/page/file/937501/download

You will see in this text references to Corporate Compliance Officers (CCOs), Compliance Committees, Compliance Officers (CO), Billing and Reimbursement Compliance Officers, HIPAA compliance officers, etc. Some organizations have

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multiple levels of compliance professionals to oversee their policies, procedures, personnel, and compliance functions. Just as the compliance plan must fit the organization, so must the duties of the necessary compliance professionals. Each must know his or her responsibilities and know how to respond to compliance issues within his/her scope of practice.

There are at least 10,000 compliance professionals in the United States, and the profession is growing every day. A complex web of healthcare laws and regulations touches all aspects of care delivery and payment. The aging population and scarcer resources put even more pressure on the healthcare delivery system.

Although compliance can be compelled where necessary, it is primarily dependent on voluntary adherence by healthcare practitioners, suppliers, and the public. CMS' effort to achieve compliance is directed toward promoting a clear awareness and understanding of the program. When efforts for achieving voluntary compliance fail, formal enforcement actions (civil monetary penalties, assessments, exclusion from the program) are taken.

Section Review 1.2

1. An effective compliance program can _____.
   a. keep a provider from facing criminal penalties
   b. make sure a practice or medical organization is 100 percent compliant with federal regulations
   c. help create financial success, customer loyalty, community support, and employee satisfaction
   d. require starting fresh with new policies and procedures and expensive changes

2. In order for a compliance program to be effective, it will need to be _____.
   a. reviewed daily for any compliance updates
   b. regularly reviewed and updated
   c. reviewed by healthcare lawyers
   d. reviewed weekly for any compliance updates

3. How many elements are required to have a successful compliance program?
   a. Five
   b. Seven
   c. Eight
   d. Nine

4. In what year did the OIG post guidance for hospitals?
   a. 1996
   b. 1997
   c. 1998
   d. 1999

5. Which entity below could NOT bill for medically unnecessary services?
   a. Hospitals
   b. Physicians
   c. Ancillary providers
   d. Patients

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Fraud and Abuse Resources

Resource	Website
HHS	http://www.hhs.gov
CMS	http://www.cms.gov
HEAT Task Force	http://www.stopmedicarefraud.gov/aboutfraud/heattaskforce
OIG	http://oig.hhs.gov
"CMS Electronic Mailing Lists: Keeping Healthcare Professionals Informed" fact sheet	http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/MailingLists_Fact-Sheet.pdf
CMS Fraud and Abuse Products	http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/Fraud-Abuse-Products.pdf
CMS Fraud Prevention Toolkit	http://www.cms.gov/Outreach-and-Education/Outreach/Partnerships/FraudPreventionToolkit.html
Frequently Asked Questions (FAQ): Medicare Fraud and Abuse	https://questions.cms.gov/faq.php?id=5005&rtopic=1887
"How CMS Is Fighting Fraud: Major Program Integrity Initiatives"	http://www.medscape.org/viewarticle/764791 **NOTE:** To access this program, you need to create a free account.
"Medicaid Program Integrity: Safeguarding Your Medical Identity" Products	www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/SafeMed-ID-Products.pdf
Medicare Learning Network® (MLN) Provider Compliance	http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/ProviderCompliance.html
OIG Advisory Opinions	https://oig.hhs.gov/compliance/advisory-opinions
OIG Compliance 101	https://oig.hhs.gov/compliance/101
OIG Email Updates	https://oig.hhs.gov/contact-us
"Reducing Medicare and Medicaid Fraud and Abuse: Protecting Practices and Patients"	http://www.medscape.org/viewarticle/764496 **NOTE:** To access this program, you need to create a free account.
OIG Compliance Guidance	https://oig.hhs.gov/compliance/compliance-guidance/

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Acronyms

CPCO	AAPC- Certified Professional compliance officer
ACA	Affordable Care Act
CLIA	Clinical Laboratory Improvement Amendments
CIA	Corporate Integrity Agreement
CCA	Certificate of Compliance Agreements
CPUP	Clinical Practices of the University of Pennsylvania
CMS	Center for Medicare and Medicaid Services
CDC	Center for Disease Control
CMPL	Civil Monetary Penalties Law
CMP	Civil Monetary Payment
CCO	Chief compliance officer
CO	Compliance officer
DOJ	Department of Justice
DEA	Drug Enforcement Administration
EMTALA	Emergency Medical Treatment and Active Labor Act
FDA	Food and Drug Administration
FBI	Federal Bureau of Investigation
GAO	General Accounting Office
HHS	U.S. Department of Health and Human Services
HIPDB	Healthcare Integrity and Protection Data Bank
HIPAA	Health Insurance Portability and Accountability Act
IDR	Integrated Data Repository
LEIE	List of Excluded Individuals/Entities
LEP	Limited English Proficiency
MFCU	Medicaid Fraud Control Unit
NIH	National Institute of Health
NPDB	National Practitioner Data Bank
OIG	Office of the Inspector General
OSHA	Occupational Safety and Health Act
PATH	Physician at Teaching Hospitals
PPACA	Patient Protection and Affordable Care Act
RAC	Recovery Audit Contractor
ZPIC	Zone Program Integrity Contractor

Source: http://www.cms.gov/Outreach-and-Education/Medicare-Learning-Network-MLN/MLNProducts/Downloads/Fraud_and_Abuse.pdf

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